

1. This plan belongs to:

Preferred name Joe

Date completed 31 July 2025

Full name Josiah Campbell

Date of birth 17 December 1942

Address Meadow Springs Care Home,
Shrewsbury, SY23 3XQ

NHS/CHI/Health and care number

3 4 5 5 9 8 6 7 3 2 1 9

The ReSPECT process starts with conversations between a person and a healthcare professional. The ReSPECT form is a clinical record of agreed recommendations. It is not a legally binding document.

2. Shared understanding of my health and current condition

Summary of relevant information for this plan including diagnoses and relevant personal circumstances: Joe has multiple medical conditions including heart failure, diabetes, chronic kidney disease and anaemia. These are generally well managed with oversight in his care facility and occasional input from his community medical team who know his situation well, however has required admission to hospital for management at times of crisis when one of his conditions becomes unstable. Joe is aware that his conditions can be managed with ongoing treatment but cannot be reversed and may deteriorate in time.

Details of other relevant care planning documents and where to find them (e.g. Advance or Anticipatory Care Plan; Advance Decision to Refuse Treatment or Advance Directive; Emergency plan for the carer):

No other care plans at present.

I have a legal welfare proxy in place (e.g. registered welfare attorney, person with parental responsibility) - if yes provide details in Section 8

☐ Yes ☒ No

3. What matters to me in decisions about my treatment and care in an emergency

Living as long as possible matters most to me

Quality of life and comfort matters most to me

What I most value:
At this point in my life I would like to be kept comfortable and only receive treatments which allow me to stay in my home. I am reluctant to be admitted to hospital.

What I most fear / wish to avoid:
I don't want lots of painful investigations and treatments. I don't want anyone pressing on my chest if my heart stops.

4. Clinical recommendations for emergency care and treatment

Prioritise extending life

or

Balance extending life with

comfort and valued outcomes

or

Prioritise comfort

Dr B Valentine

clinician signature

clinician signature

clinician signature

Now provide clinical guidance on specific realistic interventions that may or may not be wanted or clinically appropriate (including being taken or admitted to hospital +/- receiving life support) and your reasoning for this guidance:

Based on information recorded above:

- prioritise treatments that can be administered in the community (e.g. IV diuretics if appropriate).
- involve community heart failure team early.
- avoid invasive treatments such as organ support or CPR.
- consider assessment in emergency department in unforeseen, potentially treatable situations e.g. after a fall.
- prioritise comfort and symptom control in event of deterioration of diagnosed conditions.

CPR attempts recommended
Adult or child

clinician signature

For modified CPR
Child only, as detailed above

clinician signature

CPR attempts **NOT** recommended
Adult or child

clinician signature
Dr B Valentine

5. Capacity for involvement in making this plan

Does the person have capacity to participate in making recommendations on this plan? ☒ **Yes** ☐ **No**
Document the full capacity assessment in the clinical record.

If no, in what way does this person lack capacity?

If the person lacks capacity a ReSPECT conversation must take place with the family and/or legal welfare proxy.

6. Involvement in making this plan

The clinician(s) signing this plan is/are confirming that (select A,B or C, OR complete section D below):

- ☒ **A** This person has the mental capacity to participate in making these recommendations. They have been fully involved in this plan.
- ☐ **B** This person does not have the mental capacity, even with support, to participate in making these recommendations. Their past and present views, where ascertainable, have been taken into account. The plan has been made, where applicable, in consultation with their legal proxy, or where no proxy, with relevant family members/friends.
- ☐ **C** This person is less than 18 years old (16 in Scotland) and (please select 1 or 2, and also 3 as applicable or explain in section D below):
- ☐ **1** They have sufficient maturity and understanding to participate in making this plan
- ☐ **2** They do not have sufficient maturity and understanding to participate in this plan. Their views, when known, have been taken into account.
- ☐ **3** Those holding parental responsibility have been fully involved in discussing and making this plan.
- D** If no other option has been selected, valid reasons must be stated here: (Document full explanation in the clinical record.)

7. Clinicians' signatures

Grade/speciality	Clinician name	GMC/NMC/HCPC no.	Signature	Date & time
Senior responsible clinician: GP	Dr Bally Valentine	GMC 4123485	B. Valentine	31/7/25 14:00

8. Emergency contacts and those involved in discussing this plan

Name (tick if involved in planning)	Role and relationship	Emergency contact no.	Signature
Primary emergency contact: Clive Campbell ☒	Nephew	07843 806541	optional
Community heart failure team ☐		01248 543775	optional
GP surgery ☐		01248 641555	optional
☐			optional
☐			optional

9. Form reviewed (e.g. for change of care setting) and remains relevant

Review date	Grade/speciality	Clinician name	GMC/NMC/HCPC No.	Signature

If this page is on a separate sheet from the first page: **Name:**

DoB:

ID number: